

DRC EBOLA (BVD) OUTBREAK

Integrated Epidemiology, Mobility, Mining & Security Briefing Note

Epidemiological data through: **8 September 2026** | Security (ACLED): 8 Aug–4 Sep 2026 | Datasets: INSP sitrep, DRC health zones, Flowminder, IPIS mining, ACLED

BOTTOM LINE. Ituri remains the centre of the outbreak, but **North Kivu is now accelerating fastest** — weekly new confirmed cases there rose from 135 to 205 in the past epi-week (+52%) while Ituri plateaued (354 → 357). **Mandima–Mambasa–Komanda** are the clearest concentration of security risk inside affected zones. The information below answers the following questions: *“where is transmission growing, where are exposed populations moving, and where could insecurity prevent us from detecting spread early?”*

Key messages

As of 8 September the dataset records **6,843 confirmed cases across 61 health zones in six provinces** (3,310 confirmed deaths; 1,611 recovered; 833 in isolation). Case counts alone do not describe operational risk — mobility, artisanal mining, insecurity and health-service access create transmission and surveillance vulnerabilities beyond the main hotspots. Three geographies need attention:

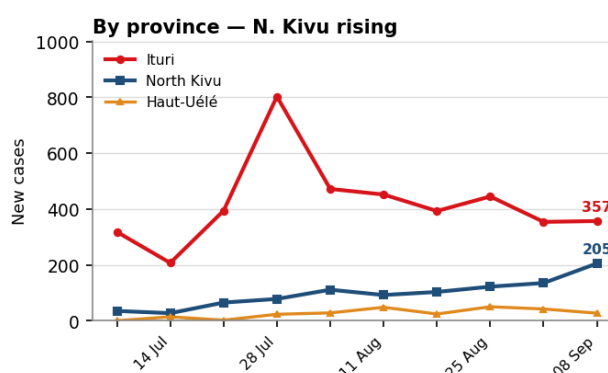
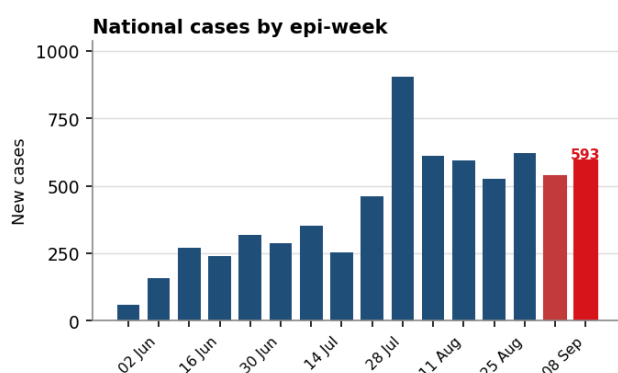
- **High disease burden:** Bunia, Rwampara, Nizi, Mongbwalu (Ituri).
- **Rapid epidemiological growth:** Katwa, Beni, Butembo (North Kivu).
- **High access / security risk:** Mandima, Mambasa, Komanda, where recent conflict overlaps affected zones.

1. Epidemiological situation & weekly trend

National weekly incidence rose to 593 new confirmed cases in the week to 8 September, up from 537 the week before. The acceleration is driven almost entirely by North Kivu; Ituri's weekly incidence has flattened. Week-on-week by province:

Province	New — wk to 1 Sep	New — wk to 8 Sep	Δ wk/wk	Cumulative 8 Sep
Ituri	354	357	+3	5,461
North Kivu	135	205	+70	1,093
Haut-Uélé	42	27	-15	258
National	537	593	+56	6,843

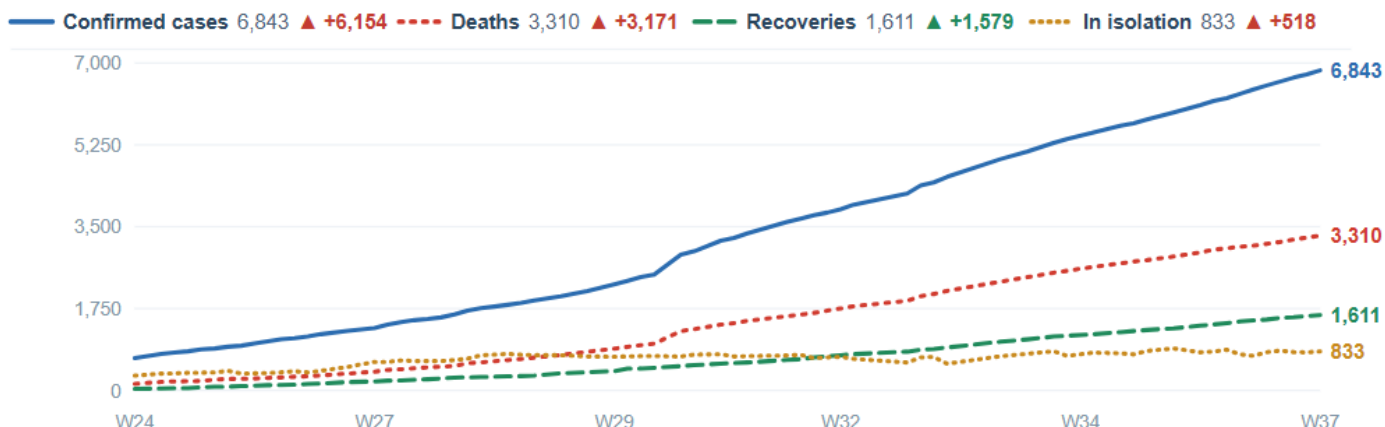
Largest weekly health-zone increases (to 8 Sep): Bunia +110, Katwa +69, Nizi +53, Beni +39, Butembo +37, Rwampara +35, Mangala +34, Komanda +26. The **Katwa–Beni–Butembo corridor** added +145 in one week and, linked by established population movement, should be treated as one connected transmission geography.



Weekly figures are new confirmed cases per 7-day epidemiological week, derived from the INSP cumulative daily series (some reporting-day gaps carried forward). The 28 Jul peak partly reflects catch-up reporting after mid-July gaps.

National indicators over time

Reported cumulative people · through 2026-W37 · cut-off 2026-09-11 · epi weeks (ISO-8601) · gaps left missing



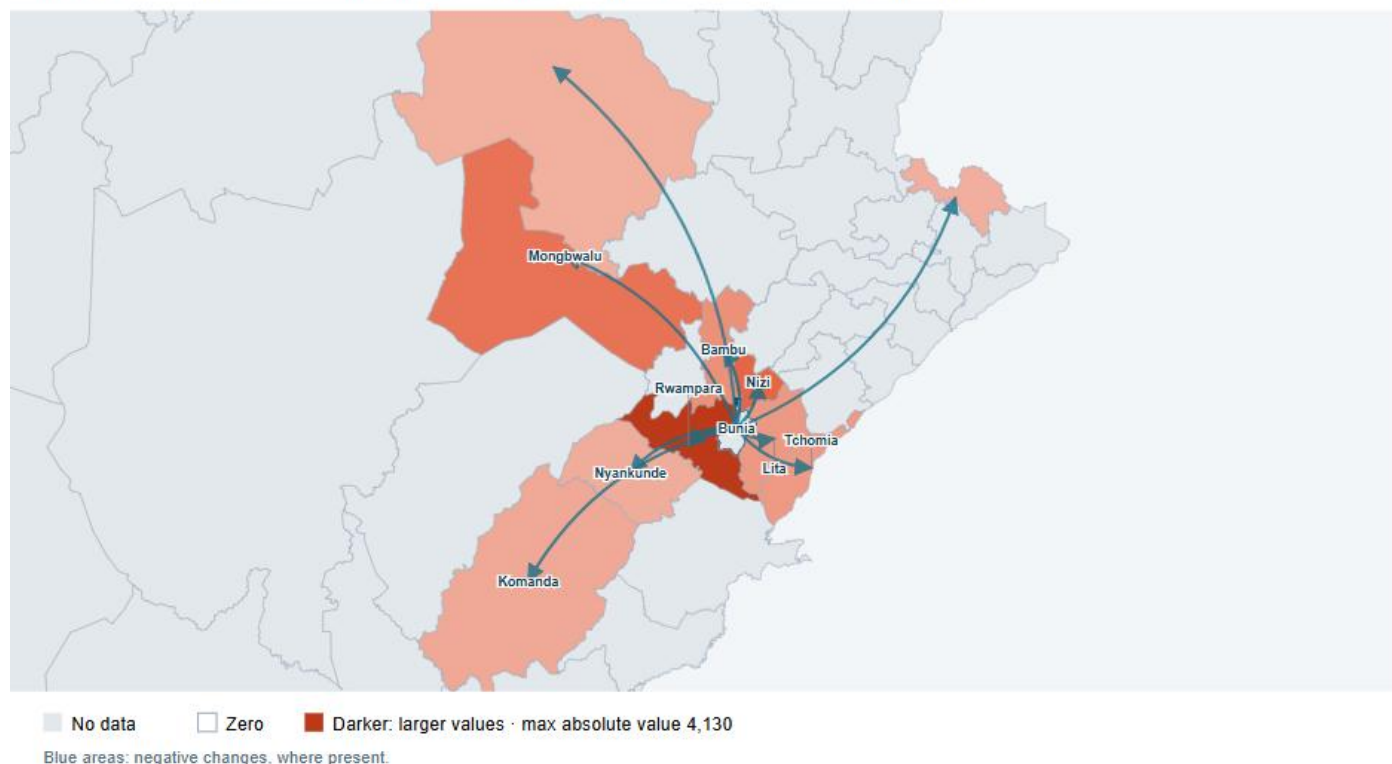
Source: https://raw.githubusercontent.com/INRB-UMIE/BDBV2026-Data/main/data/insp_sitrepreprocessed/insp_sitrepreprocessed_national_cumulative_confirmed_cases_daily.csv · Rising cumulative totals reflect

2. Mobility widens the operational picture

Flowminder shows populations from affected zones moving across a wider network than the case map alone. Cohorts in Bunia, Mongbwalu and Rwampara reach receiving zones including Lita, Nizi, Bambu, Tchomia and Komanda; expanded origins now include Beni, Butembo and Katwa. Mobility does not prove transmission — it flags where surveillance may need strengthening *before* cases appear. Use it as a connectivity/prioritisation layer, not traveller counts.

Outflow from Bunia

health_zone · directed mobility · estimated relocations (people) · cut-off 2026-09-11; observation dates may differ



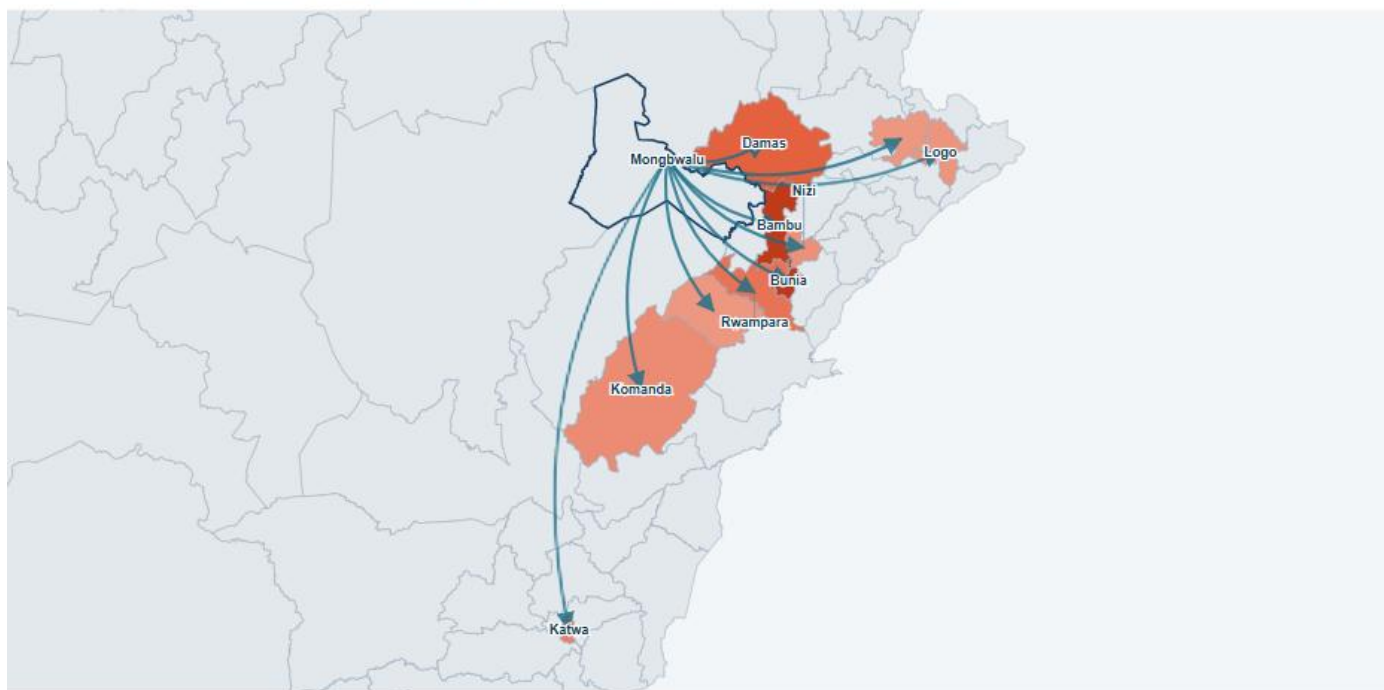
Teal arrows show destinations from this area — places to assess for onward surveillance.

Arrowheads point from origin to destination. Curves connect representative area centres; they do not show actual travel paths. Internal movements are excluded.

Origin	Destination	estimated relocations (people)	Outbreak context (2026-09-08)
Bunia	Rwampara	4,130	999 cumulative cases; seven-day change 35
Bunia	Nizi	1,656	723 cumulative cases; seven-day change 53
Bunia	Mongbwalu	1,293	640 cumulative cases; seven-day change 25
Bunia	Bambu	606	176 cumulative cases; seven-day change 11
Bunia	Lita	471	247 cumulative cases; seven-day change 23
Bunia	Tohomia	449	63 cumulative cases; seven-day change 6
Bunia	Komanda	246	118 cumulative cases; seven-day change 26
Bunia	Nyankunde	199	125 cumulative cases; seven-day change 1
Bunia	Mahagi	184	4 cumulative cases; seven-day change 0
Bunia	Watsa	169	Case data unavailable

Outflow from Mongbwalu

health_zone · directed mobility · estimated relocations (people) · cut-off 2026-09-11; observation dates may differ



■ No data □ Zero ■ Darker: larger values · max absolute value 1,244

Blue areas: negative changes, where present.

Teal arrows show destinations from this area — places to assess for onward surveillance.

Arrowheads point from origin to destination. Curves connect representative area centres; they do not show actual travel paths. Internal movements are excluded.

Origin	Destination	estimated relocations (people)	Outbreak context (2026-09-08)
Mongbwalu	Bunia	1,244	1,523 cumulative cases; seven-day change 110
Mongbwalu	Bambu	1,215	176 cumulative cases; seven-day change 11
Mongbwalu	Damas	544	32 cumulative cases; seven-day change 4
Mongbwalu	Rwampara	386	999 cumulative cases; seven-day change 35
Mongbwalu	Katwa	290	477 cumulative cases; seven-day change 69
Mongbwalu	Komanda	212	118 cumulative cases; seven-day change 26
Mongbwalu	Nizi	183	723 cumulative cases; seven-day change 53
Mongbwalu	Logo	160	13 cumulative cases; seven-day change 2
Mongbwalu	Rimba	151	11 cumulative cases; seven-day change 0
Mongbwalu	Nyankunde	150	125 cumulative cases; seven-day change 1

Outflow from Rwampara

health_zone · directed mobility · estimated relocations (people) · cut-off 2026-09-11; observation dates may differ



■ No data □ Zero ■ Darker: larger values · max absolute value 3,802

Blue areas: negative changes, where present.

Teal arrows show destinations from this area — places to assess for onward surveillance.

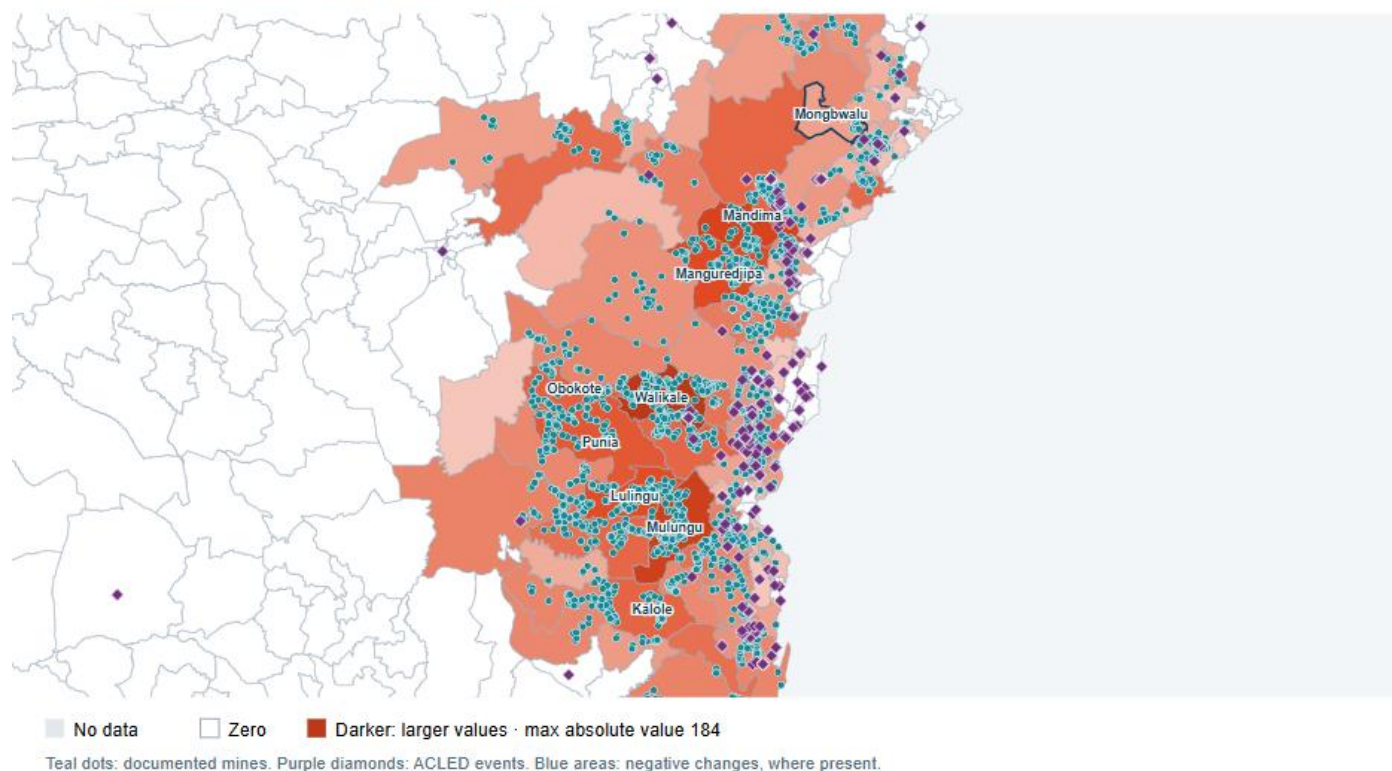
Arrowheads point from origin to destination. Curves connect representative area centres; they do not show actual travel paths. Internal movements are excluded.

Origin	Destination	estimated relocations (people)	Outbreak context (2026-09-08)
Rwampara	Bunia	3,802	1,523 cumulative cases; seven-day change 110
Rwampara	Lita	1,070	247 cumulative cases; seven-day change 23
Rwampara	Tohomia	691	63 cumulative cases; seven-day change 6
Rwampara	Mongbwalu	474	640 cumulative cases; seven-day change 25
Rwampara	Nizi	471	723 cumulative cases; seven-day change 53
Rwampara	Nyankunde	352	125 cumulative cases; seven-day change 1
Rwampara	Bambu	351	176 cumulative cases; seven-day change 11
Rwampara	Gety	278	Case data unavailable
Rwampara	Komanda	212	118 cumulative cases; seven-day change 26
Rwampara	Katwa	151	477 cumulative cases; seven-day change 69

3. Mining adds a second mobility network

Documented mining sites

health_zone · derived indicator · documented sites · cut-off 2026-09-11; observation dates may differ



IPIS holds 7,163 research-visit records representing **3,423 unique mining sites**; **794** fall inside health zones reporting confirmed cases. Mining creates connected economic geographies (workers, traders, transporters, markets); the pathway to monitor is mining community → trading point → transport corridor → commercial centre → another zone or border. Mining-intensive zones with few cases are not automatically low-risk.

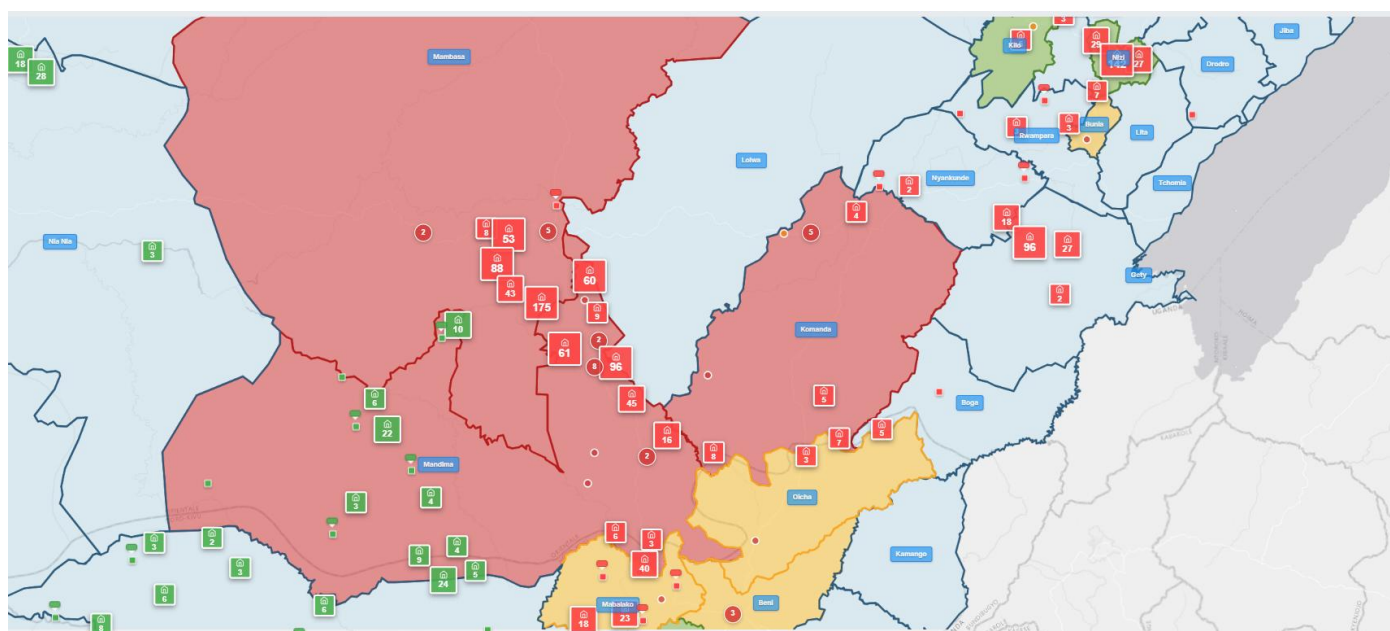
Health zone	Confirmed cases	Unique mapped mining sites
Mandima	44	137
Manguredjipa	4	118
Mambasa	17	74

Health zone	Confirmed cases	Unique mapped mining sites
Gety	7	69
Nizi	723	51
Lubero	7	42
Nia Nia	228	42
Mabalako	10	35

4. Recent conflict reveals access risks (ACLED, 8 Aug–4 Sep)

The four-week window records 269 events nationally / 176 reported fatalities; 239 events fall in the six Ebola-affected provinces. **Within affected health zones: 69 events and 73 reported fatalities.** The concentration is not in the largest-caseload zones:

Health zone	Ebola cases	ACLED events	Fatalities
Mandima	44	15	53
Mambasa	17	10	11
Komanda	118	8	3



[Legend: ■ = number of security events | ■ = # of mining sites close to the security event | ■ = # of mining sites unaffected]

These three zones account for 67 of the 73 fatalities inside affected zones. **Mandima is the clearest compound-risk case:** 44 cases (+8 in the latest week), 137 mining sites, 15 ACLED events, 53 fatalities — ADF named as Actor 1 in 12 of 15 events. Not causal, but a priority for access planning and surveillance continuity: where insecurity restricts access, low recorded case numbers may mask harder conditions for contact tracing, alert investigation and sample transport.

5. Operational implications & immediate actions

- **Where transmission is highest / accelerating:** Bunia, Rwampara, Nizi, Mongbwalu, Katwa, Beni, Butembo.
 - **Where transmission could be amplified:** Nizi, Komanda, Mandima, Mambasa, Manguredjipa, Nia Nia, Lubero, Mabalako.
 - **Where teams may struggle to see/reach transmission:** Mandima, Mambasa, Komanda — integrate security into epidemiological planning.
1. **Maintain a rolling 28-day security overlay** — refresh ACLED weekly; flag new violence over active transmission or contact tracing.
 2. **Prioritise the Katwa–Beni–Butembo corridor** — one connected transmission geography given acceleration and movement.
 3. **Access review for Mandima–Mambasa–Komanda** — check impact on alerts, contact follow-up, sample transport, referral, engagement.
 4. **Target surveillance in mining communities** — use IPIS to shortlist sites; verify which are active before deploying.
 5. **Use Flowminder to anticipate expansion** — enhance surveillance in strongly connected receiving zones before cases rise.

6. Combine datasets in one leadership view — transmission, mobility/mining, security/access and response presence, shown separately.

Data note. *Epidemiological figures verified against the INSP cumulative daily series through 8 September 2026 (national total 6,843). Weekly incidence is computed from cumulative reports with some reporting-day gaps carried forward. ACLED and IPIS reflect their stated windows: ACLED fatalities are reported estimates, not verified totals; IPIS records are historical and do not confirm current site activity. Spatial overlap identifies operational risk, not causation.*